

strategies, dates and yields are reported below. The final search was executed on 24 July 2026 and covers records available up to that date.

Databases and registries. PubMed (MEDLINE), Scopus and Google Scholar (first 200 hits, relevance-ranked) were searched, together with ClinicalTrials.gov and the WHO International Clinical Trials Registry Platform (ICTRP) for ongoing or unpublished trials. Grey literature was addressed by screening conference abstracts retrieved through Google Scholar and by examining the reference lists of all included records and of the most recent systematic reviews in the field, followed by forward citation tracking of each included record in Google Scholar and Scopus.

Search strings: PubMed: (“Platelet-Rich Plasma” [Mesh] OR PRP[tiab] OR “platelet rich plasma” [tiab]) AND (“Episiotomy” [Mesh] OR “Obstetric Surgical Procedures” [Mesh] OR “Delivery, Obstetric” [Mesh] OR “Birth Injuries” [Mesh] OR episiotomy[tiab] OR “perineal wound” [tiab] OR “perineal trauma” [tiab] OR “perineal tear” [tiab] OR “perineal laceration” [tiab]). Scopus: TITLE-ABS-KEY ((“platelet-rich plasma” OR PRP) AND (episiotomy OR “obstetric surgical procedures” OR “delivery, obstetric” OR “birth injuries” OR “perineal wound” OR “perineal trauma” OR “perineal tear” OR “perineal laceration”)). Google Scholar: (“platelet-rich plasma” OR PRP) AND (episiotomy OR “perineal wound” OR “perineal tear”). No restriction on publication date or study design was applied at the search stage. No language restriction was applied at the search stage, and non-English records were assessed with the assistance of published translations where available.

Records were included if they reported original clinical data (randomized controlled trials, non-randomized comparative studies, case series, or case reports) on PRP application for obstetric episiotomy or intrapartum perineal wound repair after vaginal delivery. Records were excluded if they addressed non-obstetric (gynecological) perineal or vaginal procedures; pelvic floor conditions without a distinct episiotomy or intrapartum perineal-trauma component (e.g., pelvic organ prolapse, stress urinary incontinence, vulvovaginal atrophy, lichen sclerosus); or non-clinical (in vitro or animal) studies. Systematic reviews were retained separately as secondary syntheses rather than as primary evidence.

Records were exported to a reference manager and de-duplicated automatically by DOI and title, followed by manual verification. Titles and abstracts were screened independently by two authors (D.B., A.-M.B.); full texts of potentially eligible records were retrieved and assessed against the criteria above by the same two authors, with disagreements resolved by discussion and, where necessary, adjudication by a third author (V.T.). Data were extracted into a standardized form covering design, setting, population and sample size, PRP preparation, comparator, outcome instruments, follow-up duration, and principal findings.

The PubMed query returned 13 records and the Scopus query 48 records; Google Scholar and citation tracking contributed a further 6 unique records not retrievable in either database, giving 67 records in total. After removal of 11 duplicates, 56 records were screened on title and abstract, of which 42 were excluded as addressing non-obstetric or gynecological indications, pelvic floor conditions without a distinct perineal-trauma component, or non-clinical work. Fourteen full-text articles were assessed for eligibility and 8 were excluded, leaving 5 primary clinical studies and 1 systematic review. The registry search identified 2 ongoing trials with no published results. The stages of the search and the records retained at each step are summarized in Figure 1.

Given the focused, hypothesis-generating purpose of this communication, the search was not registered as a systematic review protocol and does not claim full PRISMA compliance. It nevertheless follows PRISMA principles for reproducibility, explicit and complete search strings, a stated search date, duplicate screening, documented eligibility criteria, and a flow diagram, so that the retrieval can be independently verified or updated.